



ROUTINE MEDICAL CARE ADULT

Effective: January 1, 2026
Replaces: January 1, 2025

1. Scope of Practice

- 1.1. The descriptions of treatments prescribed in the following treatment protocols are intended to call special attention to the uniqueness of Santa Clara County and are in no way meant to replace the standard of medicine that providers are held to in the nation, the state, or the region. The default to all treatment otherwise not noted in these protocols is the national and state standard.

2. Baseline Vital Signs

- 2.1. Vital signs must be documented at the frequency specified in section 2.8. Documentation of assessment and reassessment of patient's condition is a standard of care and must be adhered to.
- 2.2. **Glasgow Coma Scale** on every patient and every reassessment
- 2.3. **Blood Pressure** on every patient and every reassessment
 - 2.3.1. First obtained blood pressure will be obtained manually by provider, subsequent blood pressures may then be obtained by non-invasive blood pressure (NIBP) if unit is equipped
- 2.4. **Respiratory Rate** on every patient and every reassessment
- 2.5. **Pulse Rate** on every patient and every reassessment
 - 2.5.1. First pulse rate will be obtained by palpation, only after assuring the mechanical correlation of cardiac ECG to the physical pulse will the rate on the cardiac monitor be acceptable for subsequent assessments
- 2.6. **Pulse Oximetry** on every patient and every reassessment, if the unit is equipped
- 2.7. **Temperature** on every initial assessment
- 2.8. Baseline vital signs, except for temperature, will be assessed every ten (10) minutes on stable patients and every five (5) minutes on unstable patients
- 2.9. **Blood Glucose Level** on assessment of ALOC, stroke, and suspected diabetics
 - 2.9.1. Obtained with intravenous access (IV) start or finger stick
 - 2.9.2. Readings of less than 60 mg/dl along with symptoms of hypoglycemia or altered mental status requires intervention ([700-A03](#))

3. Advanced Vital Signs

- 3.1. **Cardiac Monitoring (ECG)** on every ALS patient and every reassessment
- 3.2. **Capnography** on every patient that received an airway adjunct BLS or ALS
 - 3.2.1. No airway, BLS or ALS, will be deemed patent and/or sufficient unless there is the development of a capnographic wave form, if unit is equipped
 - 3.2.2. After any administration of **Midazolam**, patient's respirations must be monitored with capnography



4. Airway

- 4.1. **Airway Management** is defined under **700-M01**
- 4.2. **Oxygen Administration** will be titrated to achieve a pulse oximetry saturation between 94%–100%
 - 4.2.1. The provider will select the most appropriate way to deliver the oxygen, by either; nasal cannula, non-rebreather mask or bag valve mask
 - 4.2.2. Responders not equipped with pulse oximetry will titrate oxygen to the patient's relief of symptom
- 4.3. **Assisted Ventilations** will be delivered by a bag valve mask (BVM) and supplemental oxygen to the rate and volume specified when prescribed in the treating protocol.
- 4.4. **CPAP** is defined under **700-M12**

5. Circulation

- 5.1. **CPR** is defined under **700-S01**
- 5.2. **Fluid Administration** will be titrated to patient condition
 - 5.2.1. IV starts where there is no or minimal need for fluid administration TKO or a saline lock is acceptable
 - 5.2.2. If the patient needs fluid resuscitation, the amount of fluid and interval of reassessment will be outlined in the treating protocol
- 5.3. **Fluid Bolus** will be prescribed as a **250 ml Normal Saline** infusion, unless the treating protocol prescribes a different amount or frequency, the treating protocol will always take precedence

6. General Pain Management

- 6.1. Pain will be managed according to the following sections unless the treating protocol prescribes a different course, the treating protocol will always take precedence
- 6.2. Pain scale score must be documented prior to and post administration of any analgesic
- 6.3. Minor to moderate pain management or morphine allergy
 - 6.3.1. **Intravenous Acetaminophen 1000 mg** (for adult patients weighing 50 kg or more) **IV Piggyback or IV Drip**, delivered over fifteen (15) minutes. Directly puncture the vial using a macro-drip set, open drip chamber vent valve and deliver at a rate of **60 gtts/min** (dose is 15 mg/kg for adult patients weighing less than 50 kg)
- 6.4. Severe pain management or acetaminophen allergy
 - 6.4.1. **Morphine 2 mg IV**, if systolic blood pressure is greater than 100 mmHg, every 5 minutes, titrated to pain, up to 20 mg max
 - 6.4.2. OR; **Morphine 5–10 mg IM**, if systolic blood pressure is greater than 100 mmHg, may repeat in 20 minutes, up to 20 mg max
- 6.5. **BASE CONTACT**: if additional Morphine above 20 mg is needed
- 6.6. Consider monitoring patient's respirations with capnography

7. Intravenous Acetaminophen Contraindications

- 7.1. Prior acetaminophen use that exceeds 3000 mg in a twenty-four (24) hour period
- 7.2. Severe liver disease
- 7.3. Chronic alcoholism



- 7.4. Malnutrition
- 7.5. Allergy to acetaminophen
- 7.6. Patient under 15 years of age
- 7.7. History of tuberculosis treatment with Isoniazid
- 7.8. Suspected cardiac chest pain, continue to morphine (700-A08)

8. Patient Position

- 8.1. **Position of Comfort** may be used with patients that are conscious, intact gag reflex and without trauma
- 8.2. **Left Lateral position** may be used with patients with a depressed level of consciousness, decreased gag reflex and without trauma
- 8.3. **Full or Semi Fowlers** may be used in patients that are experiencing respiratory distress
- 8.4. **Trauma Positioning** is defined under (700-M11)
- 8.5. **Pregnancy Positioning** is defined under (700-A18). Do not lay patients that are twenty (20) weeks or more gestation flat

9. Patient Medications

- 9.1. Responders must either bring all prescribed medications with the patient to the receiving facility or document the medications along with dosage and frequency
- 9.2. Responders may assist patients with administration of physician prescribed devices including but not limited to, patient operated medication pumps, sublingual nitroglycerin and self-administer emergency medications devices

10. Medication Administration

- 10.1. Unless otherwise specified, pharmacological intervention indicates a need for transport to a hospital and further evaluation by a physician
- 10.2. Prior to administering any medication, ensure the right drug, right dose, right patient, and right route
- 10.3. Assess medication for expiration date, clarity, color, and intact seal PRIOR to administering the drug
- 10.4. Multi-dose vials of injectable medications are intended for multiple use on a single patient, not for use on multiple patients. They are multi-DOSE, not multi-PATIENT vials
- 10.5. Documentation shall include, at a minimum, medication name, dose, route, time of administration, and patient response (including vital signs)

11. Rapid Transportation Decisions

- 11.1. Responders should not delay the transport of a patient(s) to complete non-critical prehospital care if applicable